
MILI SUSPENSION 250MG/5ML (RASPBERRY FLAVOUR)

Name and Strength of Active Ingredient (s)

Each 5 ml contains:

Paracetamol BP 250 mg

Preservatives:

Methyl Paraben 0.1% w/v

Propyl Paraben 0.01% w/v

Product Description

Pink suspension with raspberry flavour.

Pharmacodynamics

Paracetamol is metabolised predominantly in the liver and excreted in the urine mainly as the glucuronide and sulphate conjugates. Less than 5% is excreted as unchanged paracetamol. The elimination half-life of paracetamol varies from about 1 to 3 hours. Plasma-protein binding is negligible at usual therapeutic concentrations but increases with increasing concentrations. A minor hydroxylated metabolite (N-acetyl-p-benzoquinoneimine) which is usually produced in very small amounts by mixed-function oxidases in the liver and kidney and which is usually detoxified by conjugation with glutathione may accumulate following paracetamol overdosage and cause tissue damage.

Pharmacokinetics

Paracetamol is readily absorbed from the gastro-intestinal tract with peak plasma concentrations occurring about 10 to 60 minutes after oral administration.

Indications

For fever & relief of mild to moderate pain.

Recommended Dose

1 to 5 years : 2.5 - 5 ml / dose

6 to 12 years : 5 - 10 ml / dose

Above 12 years & adult : 10 - 20 ml / dose

These doses may be repeated every 4-6 hours when necessary. Maximum of 4 doses in 24 hours.

Route of Administration

Oral.

Contraindications

It is contraindicated in patients with known hyper-sensitivity to paracetamol or any of the other ingredients/components.

Warning and Precautions

This preparation contains Paracetamol. Do not take any other Paracetamol containing medicines at the same time.

Allergy alert: Paracetamol may cause severe skin reactions.

Symptoms may include skin reddening, blisters or rash.

These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Paracetamol should be given with care to patients with impaired kidney or liver function. If there is no relief of symptoms after 3 days, please consult your physician.

Interactions with Other Medicaments

The anticoagulant effect of warfarin and other coumarine may be enhanced by prolonged regular daily use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

Pregnancy & Lactation

Use in pregnancy:

- Considered to be the analgesic of choice in pregnant patients.
- Although it crosses placenta, paracetamol is considered to be safe in normal therapeutic doses for short-term use as a minor analgesic/antipyretic in pregnancy.

Use in lactation:

- Excreted in breast milk.
- Maternal ingestion of paracetamol in normal therapeutic doses does not appear to present a risk to the nursing infant.

Side Effects

Side effects of paracetamol are rare and usually mild, although haematological reactions including thrombocytopenia, leucopenia, pancytopenia, neutropenia and agranulocytosis have been reported. Skin rashes, and other hypersensitivity reactions occur occasionally.

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

Symptoms and Treatment of Overdose

Symptoms:

Toxic symptoms include vomiting, abdominal pain, hypotension and sweating. The most serious adverse effect of acute overdose of paracetamol is a dose-dependent, potentially fatal hepatic necrosis. Clinical and laboratory evidence of hepatotoxicity may be delayed for up to one week. Major manifestations of liver failure such as jaundice, hypoglycemia and metabolic acidosis may take at least 3 days to develop.

Treatment:

In cases of overdose, methods of reducing the absorption of ingested drug are important. Gastric lavage is essential even if several hours have elapsed. Prompt administration of 50 g activated charcoal and 500 ml iced mannitol 20% by mouth, may reduce absorption. If the history suggests that 15g Paracetamol or more has been ingested, administer one of the following antidotes:

Acetylcysteine 20% i.v.: Administer intravenously, 20% acetylcysteine immediately without waiting for positive urine test or plasma level results: Initial doses of 150 mg/kg over 15 minutes, followed by continuous infusion of 50mg/kg in 500 ml 5% glucose/dextrose over 4 hours and 100 mg/kg in 1 L 5% glucose/dextrose over 16 hours; or

Oral Methionine: 2.5 g immediately followed by three further doses of 2.5 g at four hourly intervals. For a 3 year old child, 1 g methionine every four hours for four doses has been used; or

Oral Acetylcysteine 5%: 140 mg/kg as a loading dose, then 70 mg/kg every 4 hours for a total of 17 maintenance doses. If more than ten hours have elapsed since the overdosage was taken, the antidote may be ineffective.

Storage Conditions

Keep container tightly closed. Store below 30°C. Protect from light.

Dosage Forms

Oral Liquid Suspension

Pack Size

60ml, 90ml, 100ml and 120ml bottle.

Registration Number

MAL19962482XZ

Manufactured by & Product Registration Holder

KCK PHARMACEUTICAL INDUSTRIES SDN. BHD.

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